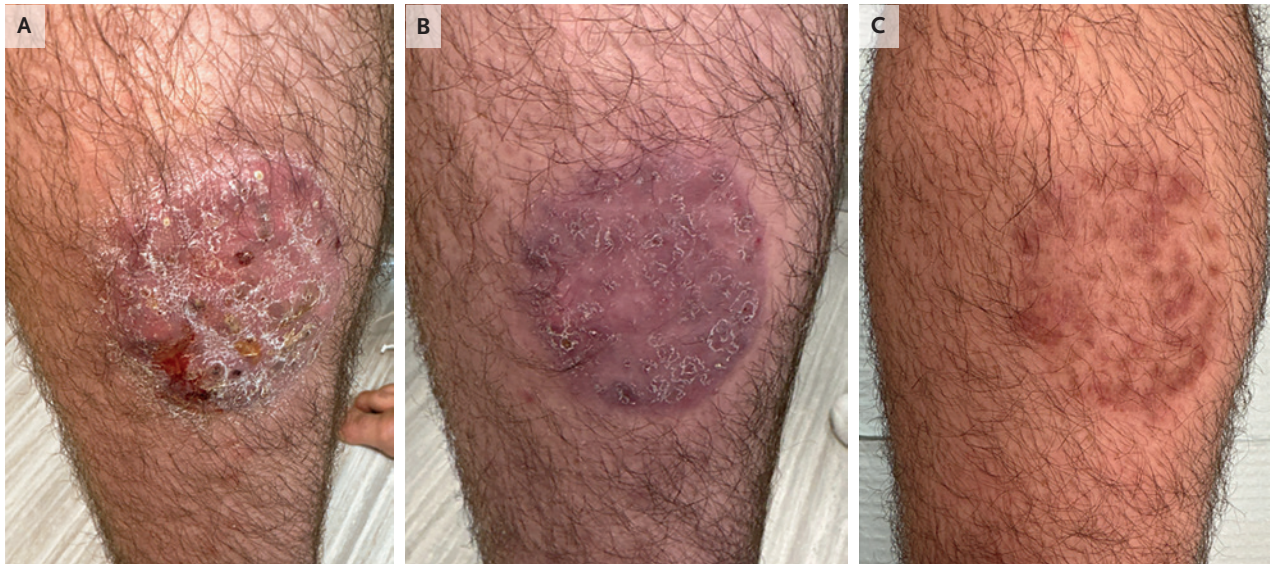


IMAGES IN CLINICAL MEDICINE

Majocchi's Granuloma



A PREVIOUSLY HEALTHY 17-YEAR-OLD HIGH-SCHOOL WRESTLER PRESENTED to the dermatology clinic with a 6-week history of an itchy rash on his calf. One month earlier, a superficial shave biopsy of the lesion performed by another provider had shown fungal hyphae in the stratum corneum. A topical antifungal agent and high-potency topical glucocorticoid had been prescribed, but the lesion had subsequently worsened. On physical examination, a circular, red, scaly plaque 6 cm in diameter with papules, nodules, and pustules was seen on the posterior left calf (Panel A). Fungal culture of a skin scraping grew *Trichophyton tonsurans*. A diagnosis of Majocchi's granuloma was made. Majocchi's granuloma is a localized deep fungal infection of hair follicles that may occur in immunocompromised patients or when topical glucocorticoids are applied to superficial fungal infections. Unlike superficial dermatophyte infections, Majocchi's granuloma is treated with systemic medications. Although the diagnosis typically requires visualization of perifollicular granulomatous inflammation on histopathological examination, a probable diagnosis was made in this case on the basis of the clinical presentation, the presence of dermal nodules, and the preceding use of a potent topical glucocorticoid in the context of histologically proven superficial fungal infection. Oral terbinafine was prescribed, and the lesion abated (Panel B, after 1 month of treatment; Panel C, 6 months after completion of treatment).

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